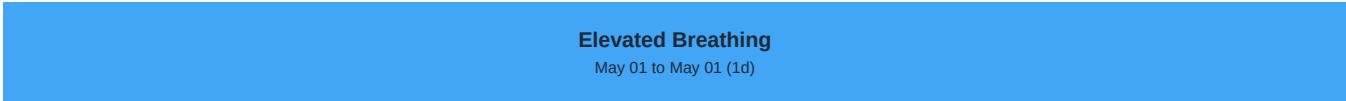


Patient ID: ThornburgPeriod: May 1 to June 1, 2026Report Date: June 2, 2026Coverage: 743/745h (100%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 32 days from May 01 to Jun 01

■ HR ■ Breathing



Last 24 Hours

Vital		Avg	Min	Max
Heart Rate		57 bpm	48 bpm	78 bpm
Breathing		17 brpm	11 brpm	34 brpm
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
3	3h	Elevated Breathing	<1	100%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 01	1 AM to 2 AM	3h	3	Elevated Breathing	24	18	29	Check SpO2 and lung sounds; assess for fluid overload or early infection

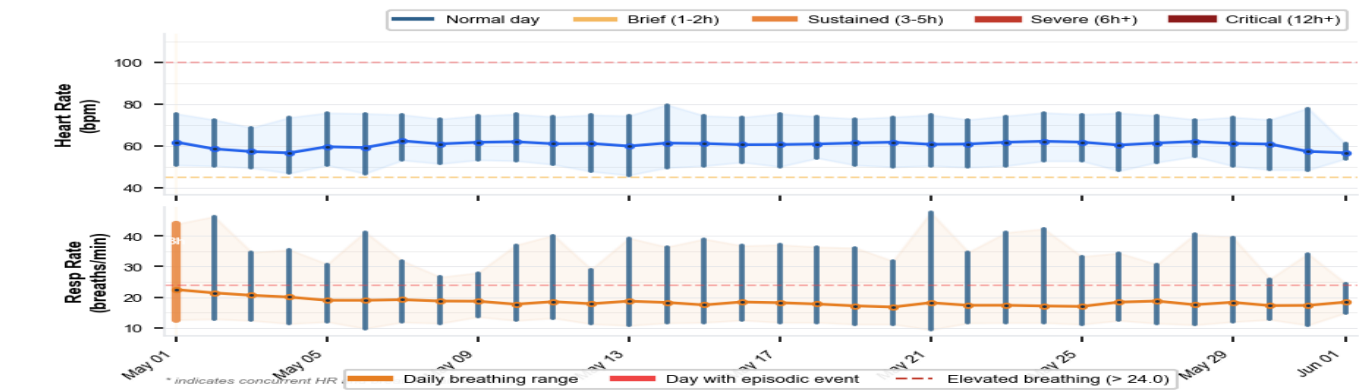
Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

Suggested Clinical Review Actions

- Elevated Breathing (avg 24 brpm, peak 29 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

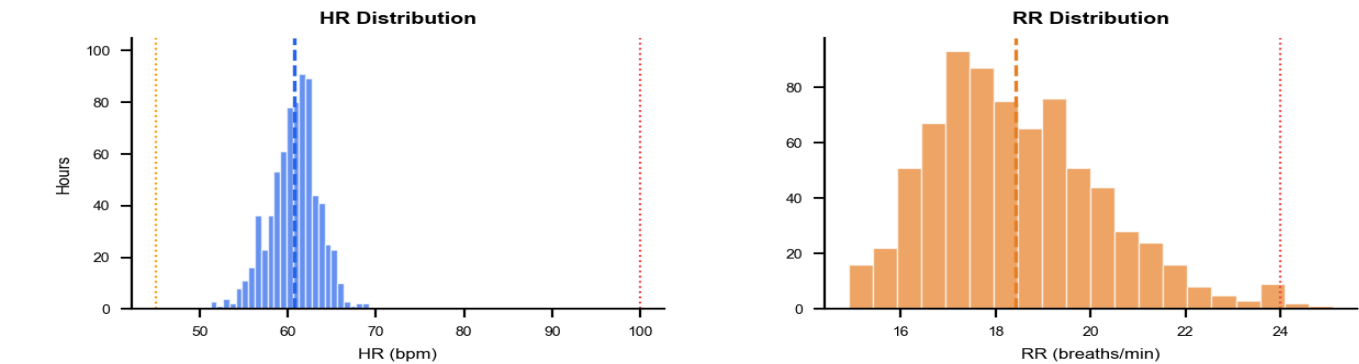
Clinical Pattern Observations:

- 1. Clustered pattern: Episodic events on 1 of 32 days (3%).
- 2. Nocturnal pattern: 2 of 3 eligible episodes during evening or night hours.

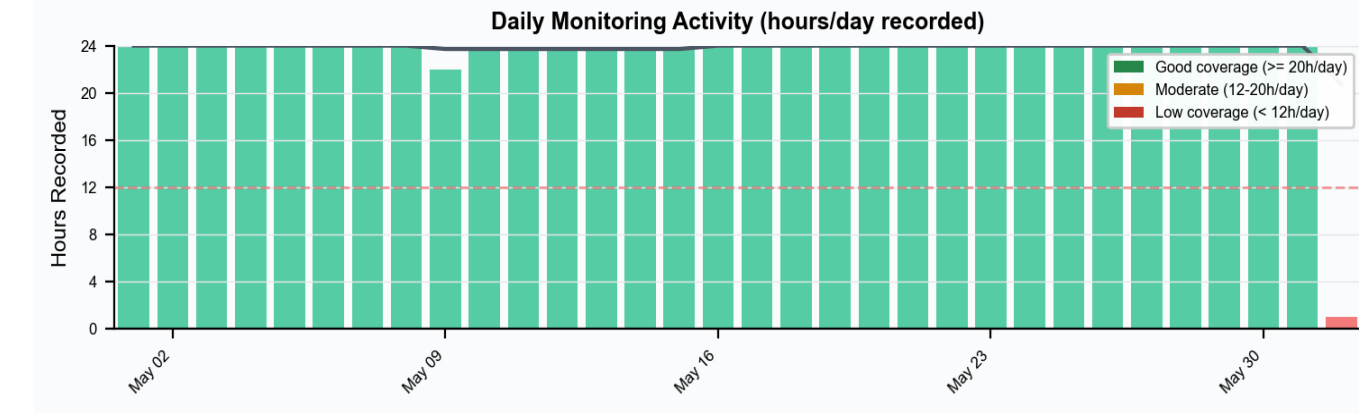


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 bpm
- High Breathing > 30 bpm
- Very High Breathing > 40 bpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.